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Project #13: Developing and implementing an Agitation Response Team (Code ART) in an urban academic emergency department

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Introduction

Introduction: Patient's arriving to triage with altered mental status, substance abuse, psychiatric emergencies and cognitive delays can often be in a heightened emotional state and display violence towards ED staff.

The current literature supports the use of behavioral emergency response teams in clinical areas, focusing on the reduction of restraint use by implementing a coordinated early response. The literature is sparse with regards to impact on employee assaults related to implementation of a behavioral emergency response team.

Current state:

- In 2022, there were 53 physical employee assaults in the ED
 - 36% of these assaults occurred within 2 hours of patient arrival to the ED.
- From January – October, 2023 there were 83 physical employee assaults in the ED.
 - 33% of these assaults occurred within 2 hours of patient arrival to the ED.

Pre-Implementation Data August 1 – October 31 2023

	Total
# of employee assaults within 2 hours of patient arrival	7
# of total employee assaults occurring in the ED	28
% of assaults occurring within 2 hours of patient arrival	25%

3 Month Implementation Data November 2, 2023 – January 31, 2024

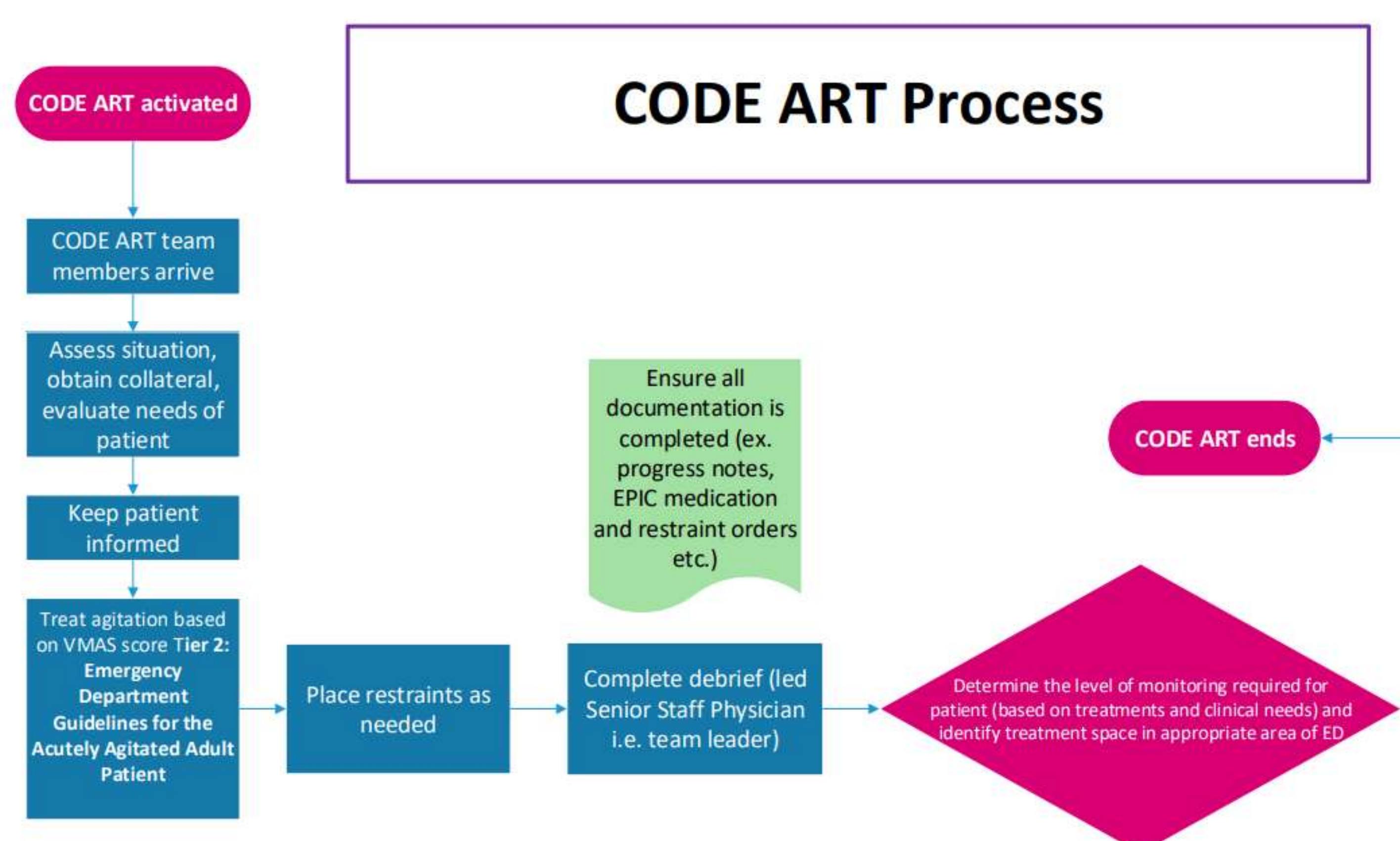
	Total
# of employee assaults within 2 hours of patient arrival	8
# of total employee assaults occurring in the ED	35
% of assaults occurring within 2 hours of patient arrival	23%

AIM

- Problem Statement:** The acute presentation of patients with psychiatric emergencies, altered mental status, substance use, and cognitive delays creates a safety risk to employees working in the triage area. A lack of standardized responses impacts response time and increase risk of injury to employees.
- Project Goal:** Implementing multidisciplinary agitation response team to provide safer care for the acutely agitated patient, promote a safer environment for ED staff, and promote dignity for the acutely agitated patient that arrives to the ED's triage area.
- AIM Statement:** Decrease the number of employee assaults occurring within 2 hours of patient arrival to HFH ED by 10% within 6 months of implementation.

Process map

Live: November 2nd 2023



Methods

Stakeholders:

Interdisciplinary stakeholders of Emergency Department physicians, nurses, technicians, security, pharmacists, residents and advance practice providers met to discuss root causes and brainstorm solutions

Analysis of root causes:

- Root causes identified by the project team and key stakeholders include:
 - Lack of a standardized approach to manage the agitated patient upon arrival to the ED
 - Lack of urgency to treat the agitated patient with same approach as a medical or trauma resuscitation
 - Safety concerns for patients and staff
 - De-prioritization of dignity of agitated patient

Proposed solutions:

- Design an agitation code response team for agitated patients in the triage area. The response team will use a standardize team approach.
 - Target areas based
 - Patient arrives to triage via DPD, being petitioned for aggressive behavior.
 - Patient self arrived in prelim, calm CC suicidal ideation, then become agitated and is escalating quickly. (ex. refusing to be undressed in triage and yelling at triage staff.)
 - Out of scope: Patients assigned to a treatment area with an assigned nurse/clinician

PDSA cycle #1

Implementation process

- Defining roles and responsibilities
- Process map (identifying current and future state)
- Developing the training
 - Educational tool kit
 - Visual aids (posters, videos)
- Resource allocation
 - Ensuring availability and access to medications and restraints
 - Agitation response team medication kit
- External liaisons and awareness
 - Informing DPD, EMS services



3 Month Implementation Process Measures

November 2, 2023 – January 31, 2024

# of code ART activations	120
Average time from patient arrival to team response (minutes)	12
Average time from patient arrival to administration of medication intervention (minutes)	23